

***Strengthening access to health
system for displaced and
conflict-affected populations***

World Health Organization

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I. Introduction

“No one should have their life prospects determined by where they are born or the circumstances into which they are forced.”

- Kofi Annan – Former Secretary-General of the United Nations

In a world where modern medicine has the power to cure, prevent, and extend life, it remains a notable contradiction that millions of people are still left even without the most basic forms of care. This isn't because solutions don't exist, but because they aren't accessible. For those forced to flee their homes due to conflict, violence, or instability, healthcare often becomes a distant privilege rather than a guaranteed right. According to the recent analysis, UNHCR¹ estimates that over 100 million people are currently displaced worldwide, each facing the issue of unmet medical needs.

Regarding conflict-affected regions, displacement is not merely the loss of home; it also disrupts the vital systems. Due to the medical facilities that may be destroyed, people in such regions face increased exposure to infectious diseases, untreated chronic conditions, maternal health risks, and deep psychological trauma. Yet, if we were to have a deeper look at the issue, behind these are human stories: of children growing up without vaccinations, of parents unable to access life-saving treatments, and of communities navigating survival in uncertainty. These people often face invisible barriers just as limiting as physical ones: unfamiliar languages, uncertain legal status, financial hardship, and, at times, exclusion or discrimination. Healthcare systems, already under strain in many host countries, are rarely designed with these matters in mind. As a result, access becomes uneven, and vulnerability deepens. In these moments, the question is no longer just about the availability of care, but also about equity.

Health is globally recognized as a fundamental human right, yet for displaced populations, this right is often limited. There are numerous global commitments which reflect an understanding that no one should be excluded from essential care. However, commitments alone do not make a difference. Without deliberate and inclusive action, the gap between principle and reality continues to widen.



The Nyang health center in Yirrol East, South Sudan

¹ [The United Nations High Commissioner for Refugees](#)

II. Definition of Key Terms

Displaced Populations

Individuals who have been forced or obliged to flee or to leave their residence, in particular as a result of or in order to avoid the effects of armed conflict, situations of generalized violence, violations of human rights or natural or human-made disasters.

Universal Health Coverage (UHC)

A global health goal, promoted by the WHO and the Sustainable Development Goals (SDG 3), that grants all people, including displaced populations, access to essential health services without financial hardship.

International Humanitarian Law (IHL)

A set of rules seeking to limit the effects of armed conflict for humanitarian reasons. It protects people who are not or are no longer participating in hostilities, such as civilians and wounded combatants, and restricts the methods and means of warfare.

Communicable Diseases

Infectious disease or illness caused by another living agent, or its products, that can be spread from one person to another, often stimulated by overcrowding, poor sanitation, and lack of vaccination. (e.g., tuberculosis, cholera, malaria)

Chronic Diseases

Long-term conditions, lasting 1 year or more, that require ongoing medical attention or limit activities of daily living or both (e.g., diabetes, arthritis, chronic respiratory diseases).

III. Background Information

In recent times, access to healthcare for displaced and conflict-affected populations has become one of the most urgent global health challenges. According to UNHCR, over 100 million people worldwide, including refugees, internally displaced persons (IDPs)¹, and asylum seekers², have been forcibly displaced due to conflict, violence, persecution, and political instability. These populations often live in conditions that severely increase their risk of illness, malnutrition, and injury, while at the same time having limited access to consistent and quality healthcare.

Healthcare systems are disrupted by conflict and forced displacement in multiple, interconnected ways. Hospitals and clinics may be destroyed or repurposed for military use, supply chains for medicines can collapse, and healthcare personnel may be injured, flee, or face severe shortages. In this context, standard healthcare services, such as vaccination, maternal and child health services, and management of chronic diseases, are often interrupted. WHO reports that displaced populations are disproportionately affected by preventable infectious diseases. These medical challenges are further intensified by overcrowded camps, limited access to clean water and sanitation, and poor nutrition, often causing outbreaks of communicable diseases, as in the case of cholera outbreaks in conflict-affected areas of Yemen and measles epidemics in refugee camps in Sudan. Simultaneously, chronic conditions such as diabetes, hypertension, and cardiovascular disease are often neglected due to the focus on emergency care, leading to preventable morbidity and mortality. Mental health conditions, including post-traumatic stress disorder (PTSD), depression, and anxiety, are also highly common, but they frequently go untreated due to the same reason as the aforementioned pathologies or due to the lack of mental health services.

The Breakdown of Healthcare Systems in Conflict Areas

Armed conflict continues to be one of the primary catalysts of both displacement and healthcare system collapse. Over time, international humanitarian law (IHL) has attempted to protect medical services, notably through the Geneva Conventions, which require the protection of medical personnel and facilities during war. Despite this, conflicts have increasingly involved damage to healthcare infrastructure, turning medical systems themselves into points of vulnerability.

In countries such as Syria, Yemen, and Afghanistan, repeated attacks, prolonged instability, and lack of maintenance have left hospitals partially functioning or completely inaccessible, especially in rural or high-conflict areas. This situation forces populations to rely on limited or informal care, often far from their home. As a result, healthcare systems lose their capacity to provide consistent and preventive care, thereby concentrating resources solely on the most critical cases. Furthermore, this situation weakens disease monitoring and early warning systems, making it significantly harder to detect and manage outbreaks before they spread.

¹ [IDPs](#)

² [Asylum seekers](#)

Major Medical Challenges and Global Responses

Displaced populations face a wide range of medical challenges that have evolved over time. Communicable diseases such as cholera, measles, and tuberculosis remain a major concern, particularly in overcrowded environments with limited sanitation, as in conflict-affected regions. Limited vaccination coverage and disrupted disease surveillance systems further worsen these risks, making early detection and response significantly more difficult.

With the adoption of the Sustainable Development Goals in 2015, particularly Goal 3 on health, and the promotion of Universal Health Coverage (UHC), there has been increased recognition that all populations, including displaced groups, should have access to essential healthcare services. However, implementation remains uneven, as the majority of displaced people have extremely limited access, and as the “non-emergency” pathologies, such as chronic diseases and mental health conditions, are often neglected in crisis settings.

Barriers to Healthcare Access and Inequality

Even when healthcare services are available, displaced populations often face multiple barriers that limit access to care, causing delayed treatment, reduced use of preventive services, and overall poorer health outcomes. Legal status is a major challenge, as individuals without proper documentation may be excluded from national healthcare systems or limited to emergency services. Financial constraints also play a significant role, with many unable to afford treatment, particularly in countries without fully subsidized healthcare. Additionally, language barriers, cultural differences, and lack of information further restrict access.

The situation varies significantly across countries. Some states, such as Germany, Canada, and France, have developed more inclusive healthcare policies that allow displaced populations access to primary and, in some cases, specialized healthcare services. These systems often emphasize early intervention and preventive care, which can reduce long-term health costs and improve public health outcomes. In contrast, countries such as Pakistan, Iran, and Egypt host large numbers of refugees but face structural limitations, including underfunded health systems, shortages of medical personnel, and high patient demand. As a result, displaced populations in these regions often rely on humanitarian organizations or informal healthcare providers, which may not be able to offer consistent care.

Efforts to address these inequalities have been reinforced through international frameworks such as the Global Compact on Refugees, which promotes greater international cooperation, responsibility-sharing, and improved access to essential services, including healthcare. However, the implementation of such agreements remains uneven and dependent on national policies, economic capacity, and political will. This creates a fundamental tension within global health governance: whether access to healthcare should be treated as a universal and unconditional

human right, or whether it should continue to depend on national resources, legal status, and domestic priorities.

From Emergency Aid to Sustainable Health Systems

Generally, healthcare for displaced populations has relied on short-term humanitarian interventions. However, as displacement becomes increasingly prolonged, it has proven insufficient. Long-term conflicts in Syria, Ukraine, and Sudan have shown the urgent need for long-term healthcare solutions that go beyond emergency response. These include sustained access to treatment for chronic diseases, expanded mental



Kid lying on a hospital bed in north-eastern Nigeria during the Boko Haram crisis

health and psychosocial support, and the strengthening of overall health system resilience. Countries such as Kenya, Nigeria, and South Africa frequently face challenges in balancing limited healthcare resources between displaced populations and their own citizens, leading to increased pressure on the already strained systems. At the same time, wealthier nations like Australia, Japan, and Ireland are often expected to contribute through financial assistance, technical expertise, and international cooperation efforts. This situation raises important questions about equitable responsibility-sharing and the long-term sustainability of global health responses.

IV. Major Countries and Organizations Involved

Germany

Germany has played a central role in supporting displaced populations, especially during the Syrian refugee crisis of 2015, accepting over one million refugees. Its healthcare system provides refugees with primary care, emergency treatment, preventive services, and vaccinations. Germany contributes financially and technically to international health initiatives, supporting WHO programs and medical aid in vulnerable countries. However, challenges appear: language barriers, bureaucratic delays, limited mental health resources, and strain on chronic disease care.

Nigeria

Nigeria faces significant internal displacement due to ongoing conflict with insurgent groups such as Boko Haram, particularly in the northeast. This has placed heavy pressure on healthcare systems in affected regions, where facilities are often damaged or understaffed. Displaced populations frequently live in overcrowded camps with limited access to clean water and sanitation, increasing the risk of diseases like cholera and malaria. Access to maternal healthcare, vaccinations, and chronic disease treatment remains inconsistent. Nigeria collaborates with WHO and other partners to deliver a better medical system. However, funding gaps, insecurity, and weak infrastructure continue to limit long-term healthcare access for displaced populations.

Pakistan

Pakistan hosts one of the largest refugee populations globally, including millions of Afghan refugees who have arrived over decades. This long-term displacement has placed enormous strain on the national healthcare system, especially in border regions such as Khyber Pakhtunkhwa and Balochistan. Pakistan's policies provide refugees with access to primary care and vaccination, but gaps remain in mental health services, chronic disease management, and preventive care. Overcrowding in camps and insufficient resources create tensions between host communities and refugees.

Sudan

Sudan has endured decades of conflict, political instability, and natural disasters, causing massive internal displacement and refugee influxes. The Darfur conflict (2003–present) and fighting in South Kordofan and Blue Nile have destroyed healthcare infrastructure, leaving many IDPs without essential services. Overcrowded camps, poor sanitation, and malnutrition increase vulnerability to diseases like cholera, measles, and malaria. Chronic diseases and mental health needs remain largely unmet due to damaged facilities and limited personnel. Sudan relies heavily on WHO and other organizations for medical aid and services. Progress is obstructed by funding shortages, political instability, and ongoing conflicts.

Canada

Canada has a strong record of supporting displaced populations and global health initiatives. It provides comprehensive healthcare for resettled refugees, including vaccinations, preventive care, and mental health services. Internationally, Canada funds WHO programs, emergency medical aid, and vaccination campaigns in conflict-affected regions. It also helps strengthen health systems in refugee-hosting countries like Pakistan and Libya. Canada focuses on long-term solutions, integrating displaced populations into national health systems, training healthcare workers, and improving disease surveillance and outbreak response.

United Nations High Commissioner for Refugees (UNHCR)

UNHCR protects refugees, asylum seekers, and internally displaced persons worldwide. It provides essential healthcare, including vaccinations, maternal and child care, and emergency medical support. The agency collaborates with WHO, UNICEF, and NGOs to deliver aid in vulnerable countries. It also supports health system strengthening in host countries and trains healthcare personnel. Challenges include funding shortages, overcrowded camps, and limited access in conflict zones. In Jordan, as an example, UNHCR supported the integration of refugees into national health services, providing inclusive care.



Hospitalized people amid civil war, Sudan

V. Timeline of Events

Date	Description of event
1945–1948	Post-World War II Humanitarian Framework After WW II, millions were displaced, leading to the creation of modern humanitarian systems and the establishment of the World Health Organization in 1948 to address global health needs.
1949–1977	Geneva Conventions and Protocols established protections for civilians and medical personnel during conflict, including the obligation to protect healthcare services.
1979–1989	The Soviet-Afghan War triggered one of the largest refugee crises of the time, placing pressure on neighboring countries like Pakistan and exposing gaps in healthcare access for displaced populations.
1998	Guiding Principles on Internal Displacement established standards for the protection of internally displaced persons (IDPs), including access to basic healthcare services.
2005	The International Health Regulations (IHR) strengthened global capacity to detect and respond to public health emergencies, including in crisis and displacement settings.
2015	The Sustainable Development Goals (SDGs) emphasized Universal Health Coverage and inclusive healthcare for vulnerable populations.
2018	The Global Compact on Refugees (GCR) strengthened international cooperation and responsibility-sharing, including healthcare access for refugees.
2020-2022	The Covid-19 pandemic exposed major inequalities in healthcare access, particularly for refugees and internally displaced persons, who often lacked access to testing, treatment, and vaccines due to weak health systems and restrictive legislation.
2022-present	The conflict in Ukraine triggered large-scale displacement, testing health systems across Europe and highlighting the importance of preparedness for health emergencies.

VI. Relevant UN Treaties and Events

★ UN Security Council Resolution 2286 (2016)

★ New York Declaration for Refugees and Migrants (2016)

- It emphasizes access to basic services, including healthcare, education, and shelter. This declaration laid the groundwork for later frameworks, particularly the GCR, and reflects a shift toward more comprehensive and long-term approaches.

★ Global Compact on Refugees (2018)

- GCR builds on previous frameworks and it specifically encourages the inclusion of refugees in national healthcare systems rather than relying solely on emergency aid. It also supports international cooperation, funding, and capacity-building to improve access to healthcare for displaced populations worldwide.

VII. Previous Attempts to Solve the Issues

1. Legal and Regulatory Frameworks

- There have been established several frameworks or policy guidelines to protect displaced populations and ensure access to basic services. Yet, while these frameworks exist and establish important principles, they often lack enforcement mechanisms and do not provide detailed guidance on healthcare system implementation.

2. Emergency Humanitarian Health Responses

- Organizations like the WHO and UN agencies have focused on emergency interventions, including vaccination campaigns, disease control, and provision of essential medicines in crisis settings. However, a more effective though debatable solution would be not focusing solely on emergency medicine, but also on routine/basic health services for displaced or conflict affected people.

3. Shifting from Emergency Aid to System Strengthening

- Latest approaches focus on building resilient healthcare systems in host and conflict-affected countries, though implementation remains uneven due to funding gaps and political challenges.

VIII. Possible Solutions

★ Integrating Displaced Populations into National Health Systems

- Countries can include refugees and internally displaced persons in public healthcare systems rather than relying only on emergency and temporary aid. This ensures continuous care for chronic diseases, maternal health, and vaccinations, in line with the already-existing frameworks.

★ Strengthening Primary Healthcare and Mobile Clinics

- Access to primary or “basic” healthcare services could be increased through mobile clinics in remote or conflict-affected areas, especially helping those who cannot reach hospitals. This is particularly effective in rural and camp settings.

★ Expanding Mental Health and Psychosocial Support

- Increasing funding and training for mental health services, including trauma care, counseling, and community-based support, is essential, as mental health conditions are highly prevalent but often untreated.

★ Improving Disease Surveillance and Early Warning Systems

- Strengthening monitoring systems for outbreaks (e.g., cholera, measles) allows faster response and prevention, especially in overcrowded camps and fragile health systems.

Questions Delegates Should Consider During Research

- What is my country’s current policy on providing healthcare to refugees and displaced populations? Is your country more focused on national solutions or international cooperation?
- What actions has your country taken in past crises involving displaced populations?
- Where does your country’s current healthcare response to displaced populations fall short and what reforms would your country realistically be willing to implement?
- How can supply chains for medicines, vaccines, and equipment be maintained during conflict?
- To what extent should host countries be responsible for providing healthcare to displaced populations?
- How can healthcare systems be designed to remain functional during active conflict?

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